



OFFICIAL

Adult Community Mental Health Assessment Treatment Team

Process Manual

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Authorisation

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About this Manual

This manual provides a clear framework of procedures and processes for mental health clinicians who are employed in the Assessment Treatment Team (ATT) for Armadale Adult Community Mental Health Service (ACMHS) located at Orchard Avenue Centre Armadale.

Model of Care

The East Metropolitan Health Service (EMHS), ACMHS, ATT model of service describes the function of one component of a broader mental health system.

ATT provides short term recovery focussed, specialist community mental health care services for individuals living in the Armadale Health Service (AHS) catchment areas.

This document should be read in conjunction with the [EMHS Care Coordination in Mental Health Policy](#).

Principles and Key Functions

This service is provided with a recovery orientated approach that is person centred, strengths based, collaborative and empowering. The lived experience and insights of people with mental health issues and their families are at the heart of recovery orientated culture. The service meets the National Standards (NSQHS) for mental health services and the National Practice standards for the mental health workforce and is consistently audited and reviewed alongside the broader mental health systems. ATT operates on the premise that people can and do recover from mental illness.

Following triage and assessment, ATT will facilitate the most appropriate type of care (e.g., inpatient, crisis interventions, brief interventions, ongoing community and primary care) for the consumer.

The EMHS Mental Health Adult Program principles of ATT are:

- Based on EMHS Vision and Values.
- No wrong door.
- Easy access.
- The service is responsive to all referrals.
- To reduce risk to consumers, their family, carers and staff.
- Timely mental health intervention.

The **Key Functions** are:

- Facilitation of 7 day a week access to the most appropriate mental health care.
- Provision of a centralised coordinated triage component to ACMHS.
- Ensuring ease of accessibility in a timely manner for consumers and carers for **all referrals** that lead to initial recovery planning, including relapse prevention and implementation, for consumers presenting with acute mental health needs.

- Undertakes comprehensive Mental Health assessments for consumers presenting with acute mental health needs.
- Access to service from 08:30 to 22:00 hours seven days per week.
- **Short term** mental health interventions for up to 10 weeks from PSOLIS activation.
- ATT will have strong partnerships with the Mental Health Emergency Response Line (MHERL), Emergency Departments of hospitals in their catchment, and mental health inpatient units. Clear pathways of care will be defined by local protocols, ensuring that locally defined team structure is efficient and effective in its intentions.
- Ensuring ease of accessibility in a timely manner to mental health services.
- Review of all referrals.
- Providing assessment and acute and/or short-term mental health care in the community.
- Consultation and liaison with other community services to facilitate onward referral to the most appropriate service.
- Facilitation of transfers from another catchment area to clinical teams within ACMHS or other speciality teams.
- Supporting clinical teams within ACMHS to provide consumers and carers with short term crisis care after hours.
- Facilitate mental health inpatient program admissions as required.
- Complete 7-day, post discharge follow-up of consumers referred to ATT
- Prevention of inpatient mental health admissions through assertive intervention in the least restrictive environment, minimising risk and as close to home as clinically possible.
- Reduced length of stay in a mental health inpatient bed through early discharge with additional support after business hours for seven days.

The ATT will be able to:

- Provide information and advice to external health care providers on the provision of mental health care.
- Establish effective, collaborative partnerships with other mental health services, local Emergency Department services, external service providers and agencies (e.g., General Practitioner, the WA Police service, St John's Ambulance Service, Wilson One Transport, and Drug and Alcohol services).
- The Triage Officer is the first point of contact for the consumer who requests a mental health service and is not currently engaged with mental health services. The Triage Officer prioritises referrals to assist with timely intervention, identifies a triage category and ensures that the referral can be tracked initially by identification of the nominated Care Coordinator who will provide the initial assessment.

Access

ATT provides triage and a multidisciplinary mental health service to consumers with significant and short-term mental health care needs that can be safely maintained in a community setting.

ATT maintains a 'no wrong door' philosophy but has inclusion and exclusion criteria to guide consumers to the most appropriate care pathway.

Entry criterion for ATT referrals:

- Aged between 18 to 65 years; and
- Have significant mental health issues; and
- Not responding well to treatment for mental health issues by their General Practitioner; and
- Presenting with significant levels of distress and/ or psychosocial stress due to their illness that has a significant impact on their functioning; and
- Require co-ordination of care and support across multiple agencies, family, friends, support persons, carers and providers; and
- Would benefit from a multidisciplinary team review.

ATT is not suitable for:

Primary reason for referral is assessment and/ or treatment of:

- Attention Deficit Disorder / Attention Deficit Hyperactivity Disorder - redirect to private psychiatrist who specialises in ADD/ADHD assessment and treatment.
- Intellectual Disability - redirect to the GP who can refer to a Neurosciences Unit or a private neuropsychology service.
- Autism Spectrum Disorder - redirect back to the GP who can refer to the Autism Association Diagnostic Clinic, or to a private psychiatrist/psychologist who specialises in in ASD assessment.
- Continued prescription of a Schedule 4R/8 drug or medicinal cannabis - directed back to the GP with a recommendation to involve an AoD service if required.
- Medical assessment for a Disability Support Pension application - direct back to the patient's GP.
- Complete a driver's license assessment - direct back to the consumer's GP
- Psychiatric report to evidence a workers' compensation claim, fitness to work, or court proceedings – redirect to private psychiatrist specialising in providing such reports.
- Psychological treatment with no indication of requiring multidisciplinary team input, including medical review – redirect to patient's GP suggesting a Mental Health Care Plan is completed and referral to a private psychologist or suitable community organisation.
- To gain access to medical detox or a rehabilitation service for treatment of addiction - direct to an AoD service.

- Primary NDIS application requests - redirect to consumer's general practitioner
- Parenting capacity assessment report requested by CPFS- redirect to private psychiatrist specialising in providing such reports.
- CRHTT Referrals: When a consumer is referred to ATT from another service primarily because of being on a CHRTT waitlist, whilst not excluded, consideration should be made as to the level of support required prior to discharge as CRHTT provides inpatient type level care.

AMHS Catchment Areas:

Armadale	Forrestdale	Kenwick	Oldbury
Bedfordale	Gosnells	Keysbrook	Orange Grove
Brookdale	Harrisdale	Langford	Piara Waters
Byford	Haynes	Maddington	Roleystone
Camillo	Hilbert	Mardella	Serpentine
Canning Mills	Hopeland	Martin	Seville Grove
Canning Vale	Huntingdale	Mount Nasura	Southern River
Cardup	Jarrahdale	Mount Richon	Thornlie
Champion Lakes	Karragullen	Mundijong	Wungong
Darling Downs	Kelmscott	Oakford	Beckenham

Hours of Operation

ATT will operate from 08:30 to 22:00 hours, 7 days a week to meet community needs. Crisis assessments presenting outside of operating hours will generally be seen by 24-hour services such as acute clinical services, hospital emergency departments and/or mental health inpatient services. Community care team services will provide comprehensive Treatment Support and Discharge Plan (TSDP) crisis plans to ensure there is a shared understanding of consumers' needs by 24-hour services.

Clinical Governance

ATT Clinical Intake Meeting is held daily. ATT triage, ATT medical, CTT medical, and a CTT representative may attend to discuss suitable referrals for CTT. An ATT team doctor must be present at the meeting. The purpose of the meeting is to:

- Review all referrals from the previous 48hrs.
- Consumers in referral to be allocated a Care Coordinator and team doctor or outcomes to other services if not admitted to ATT.
- Discuss referrals which are identified as requiring CTT.
- Discuss discharges from the team.
- Review all consumers at 5 weeks after they have been activated into the service.
- Discuss consumers who 'do not attend' (DNA) and have not been contactable within 48hrs to develop a plan for follow up.
- Clinicians may seek advice regarding the care of a consumer and/or arrange handover of care etc. after hours.

Consumers presented at clinical intake will have a record kept in the clinical intake diary. This will take the form of a UMRN identification sticker and notation of the outcome decision. The outcome decision will also be recorded on the PSOLIS Referral outcome form (PAKMR6.1). Triage will maintain a record of consumers in referral; for presentation to intake; and outcome status of consumers. This will be maintained in the Triage handover document found in W: Drive. All data recorded into the handover document is to be in ISOBAR format. This document is accessible to all ATT staff and acts as the handover document to staff who are not on duty at the time of the clinical intake meeting.

Consumers discussed at clinical intake will have an entry recorded in the Digital medical record (DMR) outlining the clinicians in attendance, discussions, outcomes, and allocated clinicians responsible for ongoing care. After the clinical intake meeting triage will send an email to allocated clinicians. The Clerical Officer will transfer the information onto PSOLIS under Intake Meeting for each consumer discussed and includes all staff who were present as attendees.

Documentation Requirements

ATT follow the EMHS Adult Community Mental Health Services Triage to Discharge policy framework to inform clinical documentation requirement.

[State-wide Standardised Clinical Documentation for Mental Health Services](#)

ATT Triage

ATT triage is the single point of entry to AKG Adult Community Mental Health Service.

Referrals

Referrals are defined as:

Referral is a request to access services for specialised mental health intervention from an individual, community and non-government agency or any health-related service provider.

Referrals are received via fax, email, telephone, or self-presentation to the clinic. Every referral is reviewed by the Triage officer and prioritised according to urgency utilising the Triage Rating Scale. [CTRS framework](#)

Fax: 9394 0845

Fax machine in OAC clerical area and monitored by clinic clerks, triage staff and after-hours clinicians.

Telephone: 9398 6600 weekdays 08:30 to 16:30

This is the clinic number, and calls are passed onto triage through clinic clerks. Outside of normal business hours (weekdays 16:30 to 22:00; and weekend/public holidays 08:00 to 22:00) callers will be advised by recorded message to call the Mental Health Emergency Line (MHERL) who will screen calls and contact ATT directly on the ATT after-hours mobile phone **0409 125074** or by email (below).

ATT is unable to accept self-presentations to the clinic after-hours, and these should be redirected to the nearest hospital emergency department if indicated by an urgent Triage Rating Scale (CTRS) outcome.

Email: ATTReferral.Armadale@health.wa.gov.au

The email account is accessible by all ATT staff. During office hours and after-hours 16:30 to 22:00 it is monitored by clinicians on duty.

Role of Triage Clinician

- Ensure there is adequate information included in the referral for a triage assessment to be completed.
- Seek further information from the referrer if required.
- Prioritise referrals according to risk and the CTRS
- Complete a triage assessment of referrals including completing a Triage and Risk Assessment Management Plan (RAMP) on WebPSOLIS
- Respond to incoming phone calls.
- Manage the workload of triage ensuring that triages are being responded to in a timely manner according to the CTRS. This may include a request for all ATT clinicians on duty to assist and complete triage assessments.
- Complete triage assessments.
- Inform allocated clinicians to complete 7-day post discharge follow-up when Inpatient service referral for ATT care coordination is declined by clinical intake meeting.
- Provide feedback and plans to the Daily Clinical intake meeting.
- Communicate with referrers regarding outcome of referrals.
- Provide assistance to consumers in absence of care coordinator.
- Adhere to performance and compliance monitoring requirements of EMHS
- Has completed a clinical orientation to the triage role and mandatory online triage training. [AKG MSA Online Triage Training](#)

Role of Clinic Clerk

- Notifies Triage clinicians of consumer self-presentations for further advice and/or assessment as required.
- Transfers calls to Triage clinicians requiring further clinical advice and/or assessment as required.
- Takes phone messages and sends relevant demographic and context details to Triage clinicians via Triage Referral email when staff unavailable to take calls (not inclusive of urgent calls which are escalated to the next available ATT staff member to take).
- Provides clerical support to Triage in way of:
 - Responding and actioning face-to-face enquiries from Triage.
 - Responding and actioning email requests from Triage.
 - Collating and preparing referral packs (hard copy referrals that typically come through via email or fax) and delivering this to Triage.

- Booking required medical appointments for consumers via patient information systems.
- Facilitating other data entry requirements via patient information systems for clinical DMR reporting functions.
- Assisting with 7-day follow up letters processes (collate for postage and scanning copies into the DMR).
- Ordering and maintaining required forms and stationery for Triage use.
- Other clerical duties as directed by Clerical Coordinator in accordance with Triage and service needs.
- Once a triage is completed and processed through the clinical intake meeting. Triage will update the PSOLIS Referral Outcome form (PAKMR6.1). The clinic clerk will be directed by the information on (PAKMR6.1) and will outcome the referral on PSOLIS.

Triage Assessment Process

The triage assessment can be completed by telephone; Videocall (Health Direct) or face to face consultation. The purpose of the triage assessment is to gather information to identify the level of acuity, level of risk and the healthcare needs of the consumer and to formulate a plan for further assessment.

The triage assessment must include to completion of:

- State-wide Standardised Clinical Documentation (SSCD) RAMP.
- Completes the Triage field on WebPSOLIS or standardised documentation SMHR900 if PSOLIS is offline.

Key areas of a triage assessment are:

- Reason for the referral.
- Identifying the source of the information for the referral.
- Demographics (identification; address; next of kin)
- Confidentiality and consent issues
- Consumer rights
- Communication Issues.
- History and Collateral
- Medical Issues
- Current Treatments
- Drug and Alcohol use.
- Current functioning and supports
- Forensic Issues and Legal Status
- Domestic & Family Violence Assessment
- Summary and formulation of overall clinical impression.
- Action Plan
- The level of acuity with updated CTRS.
- Alerts/risks.

On completion of the triage assessment the clinician is required to formulate a plan of action which can include:

- Immediate crisis response through police, ambulance (CTRS A).
- All urgent referrals rated CTRS A, B or C shall be dealt with in accordance with:

- [EMHS Mental Health Patient Flow and Bed Capacity Framework](#)
- [EMHS Community Home Visiting Policy;](#)
- [AKG Community Home Visit Safety Guideline](#)
- [Interview Room Use Orchard Ave.](#)
- [EMHS Lone Working guide](#)
- Home visit or booked clinic appointment for assessment of mental state and risk.
- Presentation to Clinical intake meeting for:
Allocation of a Care Coordinator and treating doctor for further assessment and possible short-term treatment and management.
- Seek specialist advice from the MDT including requesting team doctors to provide General Practitioner Liaison (GPL) service to Primary care.
- Advice to consumer or carer regarding more appropriate services such as Alcohol and other drug services or Community Psychology.
- Provide a letter to consumer with recommendations.
- Provide a letter to GP with recommendations.
- Refer to other community health services such as Head to Health, or Headspace.
- Outcome of referrals which do not meet ATT entry criteria.

At times when the triage officer is unable to contact the referred consumer or Next of Kin/Personal Support Person (NOK/PSP). A decision based on the risks and evidence available is then presented by the clinician to Clinical intake meeting as to whether:

- A home visit is indicated to further assess the consumer.
- Additional contact with NOK/PSP is required.
- A letter and Esendex SMS requesting contact in the next 7 days to be sent. If contact is unable to be made with the referred person, the referral is to remain open for a period not exceeding 2 weeks from the date referral received.
- The referral will be outcomed on PSOLIS as 'Client Did Not Engage'.
- The Intake meeting will determine if further attempts to contact the consumer and NOK/PSP during the 7-day period is required. Decision making is based on known risks and is documented in the DMR.

Unless a more urgent response is indicated (CTRS B, C, and D) the triage clinician will attempt to contact the referred person and obtain as much relevant information to support the referral within a 72-hour period following receipt of the referral. All referrals will be presented to clinical intake no later than three working days from receipt of referral, or earlier if indicated (CTRS B, C, and D). Presentation to clinical intake may not be possible after 72 hours when the referral period covers weekends and public holidays. In these instances, review should occur with the Consultant Psychiatrist on-call if an urgent response is required.

Consumers attempting to contact Triage by telephone may be required to request callback when Triage clinicians are unable to receive a call. In these instances, the call will have been taken by a clinic clerk and the CTRS cannot be formulated.

Phone messages will be logged on the ATT ISOBAR handover document.

A single attempt is to be made to return calls within 72 hours. Outcomes are to be documented in the DMR, and an Esendex SMS left for the consumer if there is no answer.

Senior clinical staff will review each morning where no attempt has been made to callback within 72hrs. A plan of response will be implemented for actioning that day.

During office hours, where referrals are received for consumers aged below 18 years and over 65 years, the referral will be re-directed to the appropriate service with consideration to any immediate risk factors.

If a referral is received with an address outside of the AMHS catchment areas, the referrer should be notified on receipt of the referral and provided with the correct information, so the referral can be sent directly from the referrer to the appropriate service as soon as possible.

In the event of a self-presentation from an individual who does not reside in the AMHS catchment areas, a risk assessment utilising WebPSOLIS RAMP is to be undertaken prior to any action to divert the consumer to the correct catchment mental health service.

Triage Outcomes

When a referral is triaged, a triage event on WebPSOLIS must be completed. The referral outcome is either:

Unable to Complete:

- Referral information is logged in WebPSOLIS, and the consumer could not be contacted, or is a pending contact.

Refer To Clinical Intake:

- The triage assessment is completed and reviewed at the clinical intake meeting.

Transfer of Care: Triage to CTT or Transition Team

Triage outcome may recommend the consumer to be directly transferred to CTT or Transition Team. Referrals that may fit this criteria are:

- Referral of a consumer who is an active consumer in a CTT from another Community Mental Health Team and will require ongoing CTT support to reach their recovery goals.
- A consumer whom the MDT determine will require care co-ordination beyond 10 weeks to reach their recovery goals, and meets CTT inclusion criteria.
- A consumer being transferred on a Community Treatment Order (CTO)
- A consumer who is receiving Clozapine Treatment.
- A consumer on Olanzapine depot.
- A consumer who meets the inclusion criteria for Transition Team Model of care

Triage will log the referral and collate the demographic and SSCD documentation required for handover. They will contact the consumer to ensure they are living in the catchment area. If it is deemed appropriate for the consumer to be transferred directly to CTT the case will be presented to CTT for activation.

SSCD should Include, but may not always be available from the transferring team:

- PSOLIS Triage document

- Mental Health Assessment Form (MHAF).
- HoNoS
- RAMP
- Consumer Kessler self-report (K10)
- TSDP
- Evidence of physical examination initial request from referring service but consumer can be transferred with evidence that this has been requested.
- Physical Appearance Form
- Medication profile, including clozapine protocols.
- LSP-16 (NB: protocol exclusion if not known for 3 months)
- Care Transfer Summary

Triage will make every effort to collect all the required information from the referring service. However, there are cases where the information requested has not been completed by the referring service. This should not prevent the transfer of care of the consumer in the interests of consumer centred care and continuity of care.

Triage will present the referral with information regarding source of referral, reason for referral, current treatment, current mental state, and risk to the Clinical intake meeting.

Transfer of Care: Consumer under CTO to CTT

It is the responsibility of triage to gather the transfer information of a consumer who has been accepted by a Psychiatrist from any team outside of ACMHS under a CTO. Patients referred on CTO from ACMHS inpatient services will be processed by CTT First on Call (FOC)

Role of CTT First-on-Call (FOC)

All requests for CTO from local and out of area will be forwarded to FOC who will identify a CTT consultant psychiatrist and allocate accordingly, notifying the consultant and referring practitioner by email so that clinical handover will occur.

Role of Accepting Consultant Psychiatrist

In the event of a consumer being transferred to ACMHS on a CTO the referring inpatient/community service doctor will liaise with the receiving treating community team's Consultant Psychiatrist regarding the acceptance of the consumer.

The accepting Consultant will acknowledge referral to the service by sending an email to the ATT referral email. The email should include demographics so that the consumer can be identified on PSOLIS, any information that has been gathered regarding the consumer's presentation and confirmation that the CTO has been accepted.

Role of ATT Triage

ATT Triage will accept and manage the documentation from referring services outside of ACMHS. Triage will contact the consumer or NOK to ensure that the consumer is living at the address provided and is aware of referral to ACMHS.

Once the information has been received and risk assessment completed, the referral will be presented at the next clinical intake meeting.

Risk Management

Management of the safety of consumers, their carers', their children and our staff are a priority in any assessment.

Clinicians are to document all levels of risk for each consumer in the assessment process, to decide which (if any) clinical risks the patient may be subject to and what risk mitigation strategies are required to reduce the level of risk.

Within ATT risk assessment and management is an ongoing process starting at referral through to discharge.

Read in conjunction with:

- [OAC Code Black Policy](#)
- [Interview Room Use Orchard Ave.](#)
- [EMHS Community Home Visiting Policy](#)
- [AKG Home Visiting guideline](#)
- [EMHS Lone Working Guide](#)
- [WA Health MP 0095 Clinical Handover Policy](#)

Risk Assessment of Consumers

RAMP is completed at triage, activation, clinical review, discharge, transfer of care and when there is a change in presentation.

Risk Assessment and Management of Children

When engaging with adult consumers of the service, the safety of their children or minors in their care is of upmost concern and AMHS are aware of its responsibilities of Mandatory Reporting of Child Sex Abuse by clinicians under legislation as well as its public duty to report safety concerns of children. It is our responsibility to report any concerns to the Department of Child Protection & Family Support who will undertake their own assessment and determine what action is required. The Senior Social Worker on the team provides support and advice to staff regarding reporting requirements and processes. [Mandatory reporting guide.](#)

Risk Assessment of Family, Carers and Others

ACMHS considers the safety of family members, carers, and Personal Support Persons (PSP's) a part of risk assessment. If there are concerns about the safety of others, clinicians will respond to the concern and develop a safety plan, which may include contacting police, advising people at risk and ensuring they are safe, and also consider the application of the MHA 2014. [Family and Domestic Violence guidelines.](#)

Sexual Safety

Everyone accessing mental health services has the right to be psychologically and physically safe. This includes being free of and feeling safe from, behaviour of a sexual nature that is unwanted or makes a person feel uncomfortable, afraid or unsafe.

It is important to note that some consumers accessing our service may be more vulnerable.

- Consumers with a cognitive impairment (including cognitive impairment for mental illness) are vulnerable to abuse
- Individuals who have a history of physical and sexual abuse can have a higher degree of lifetime exposure to sexual abuse and violence.
- LGBTQI+ consumers experience increased vulnerability to sexual victimisation.

As part of a comprehensive assessment, consumers are asked how they identify in relation to gender, ensuring gender sensitive approaches and staff gender preference. Clinician allocation will consider the consumers preferences alongside clinical need and staff availability.

Sexual Safety of Consumers of Mental Health Services

Staff Safety

Staff are required to consider the risk that they may face when assessing or treating a consumer or interacting with their carers or significant others. Attention should be given to previous Alerts on the PSOLIS system and the consumers current presentation, along with presence of alcohol and drug use, access to weapons and people with whom the consumer lives. A cautious approach is taken ensuring safety of staff including using police assistance to conduct urgent assessments when deemed necessary and booking external security for high-risk consumers attending the clinic.

If a consumer is considered to be high risk to staff, security can be booked in advance of their appointment through the Team Leader.

Staff are required to follow the EMHS Community Mental Health Home Visiting Safety Policy and use an external duress system whenever they are visiting a consumer. Staff are required to conduct home visits in twos, especially for initial or urgent assessments.

Staff have access to high and moderate security assessment rooms at Orchard Avenue Centre (OAC) and have access to an internal duress system. Annual mandatory training includes de-escalation training and break away techniques.

External Security Services

The AMHS recognises there is no health service security on site at OAC, and there are occasions where planned external security resources are required on site for management of specific risks in the mental health community outpatient setting.

- Discuss with MDT the need for external security for a consumer assessment, considering consumer risk assessment, review of management and maintenance of a therapeutic environment.

- Rationale for use of external security is to be documented in DMR, TSDP and RAMP.
- Consumers PSP is to be notified of any change to management plan.
- Present request to the ATT Team Leader or delegate at least 48 hours before required attendance for review and approval.
- Once approved the team leader will contact the listed external security provider:
ERS – 0412 693 406
- The ATT Team Leader is required to inform the provider of the risk involved but is not required to inform the provider of the identity of the consumer at this point.
- Team Leader or Care Coordinator to notify clinic clerk of security booking and identity of clinician/care coordinator attending the appointment.
 - Information to be included in the notification:
 - Date / Time for security attendance
 - Consumer UMRN
 - Appointment time
 - Care Coordinator and Doctor.
- When security officers arrive, the Team Leader or Care Coordinator will provide orientation, access, and explanation of required duties at OAC, ensuring adherence to least restrictive practice principles.
- Visitor swipe card to be provided at reception. Team Leader to maintain log of security attendance.
- Security will conduct external patrols around OAC while waiting for consumer attendance and after the appointment has concluded.
- Email to be sent to all staff to advise of consumer appointment and use of specific Interview Room. Interview room should not be used as a thoroughfare during appointment where practicable.
- Security Officers to remain for minimum 4 hours in accordance with ERS contract for attendance.

Care Coordination

AKG follows the [EMHS Care Coordination in Mental Health Policy](#).

Triage outcomes where consumers are identified as requiring further assessment and short-term treatment are reviewed in the clinical intake meeting. A Care Coordinator and Treating Doctor are identified and allocated. The PSOLIS referral is updated during the clinical intake meeting identifying the allocated care coordinator. Triage will send an email to allocated clinicians after the clinical intake meeting advising of allocation to their caseload.

The Care Coordinator is required to arrange an initial face-to-face assessment within 7 days of allocation. This assessment can take place either at OAC or at the consumer's place of residence depending on the needs of the consumer and assessment of risk.

If there are circumstances where this cannot be met, the rationale for this must be recorded in the DMR and reviewed at the clinical intake meeting no later than 14 days from the time of allocation. A clinical intake meeting decision to activate the consumer to the service or close the referral must occur no later than 14 days after allocation.

Post Discharge Follow-up

All consumers referred to ATT for care coordination following an inpatient admission within AKG are to receive 7-day post discharge follow-up contact. 7-day follow-up must occur within 7 days of discharge and must not occur on the day of discharge or within 24 hours post discharge.

[Post Discharge Follow-up.](#)

7-day follow-up is to be recorded in PSOLIS as a mandatory service event using the item name of Post Discharge Follow-up

Initial Assessment

The purpose of initial mental health assessment is to determine a consumer's mental health status and if there is an ongoing need for ATT and mental health services.

At the first face to face contact, the consumer will be given the following information on:

- ☐ MH Tribunal
- ☐ MH Advocacy Service
- ☐ MH Law Centre
- ☐ Complaints process, inclusive of role of HADSCO
- ☐ Out of Hours Contact Numbers & Other Support Services
- ☐ Information for personal support persons
- ☐ Nominated persons
- ☐ Receiving treatment for a mental illness
- ☐ Consent for Sharing of patient information
- ☐ Charter of MH Care Principles
- ☐ EMHS Care Coordination
- ☐ Aboriginal Liaison Officers
- ☐ Diversity/CALD

Assessment will include consultation and collaboration with the consumer's family, PSPs. The Mental Health Assessment Form (MHAF) (standardised documentation SMHR902) must be completed or updated by the clinician completing the assessment.

The assessment will be tailored and developmentally appropriate to the specific needs of the consumer. This involves the gathering, evaluation and recording of information relative to the consumer's problem(s), strengths, functional status, or situation and must include following assessment components.

Key areas of the initial Mental Health Assessment are:

- NOK or Support Person.

- Current General Practitioner.
- Communication Issues.
- History of the presenting problem.
- Past mental health history.
- Legal Issues.
- Drug and Alcohol history.
- AoD assessment and completion of the ASSIST screening assessment.
- Family medical and mental health history.
- Medical History including allergies and adverse drug reactions.
- Metabolic Screening.
- Alcohol and drug history.
- Current treatments – identify by whom.
- Developmental and personal history.
- Current functioning and supports including strengths and current needs.
- Developmental and personal history including parental status and carer responsibilities – identify children and other dependants.
- Mental State Examination.
- Initial management plan developed with the consumer after the initial mental health assessment.

At the completion of the assessment the clinician is required to complete:

- RAMP
- MHAF
- Develop a Treatment Support Discharge Plan (TSDP)
- PSOLIS crisis plan if indicated.
- Present to Clinical Intake meeting if activation to service is not recommended.

All contact with consumers, carers and liaison with community services should be documented as a service event on PSOLIS and entries should be documented in the DMR describing the intervention and plan.

Treatment Phase of care

The ATT provides short term care coordination of activated consumers for up to 10 weeks. This may be extended if the MDT believes it is in the best interests of the consumer's Recovery but should not exceed 12 weeks. The role of the Care Coordinator is outlined in the [Care Coordination Quick Reference Guide](#)

Care coordination in ATT can include:

- Ongoing assessment to develop a TSDP
- Monitoring of mental state and side effects after the commencement or changes to pharmaceutical treatment

- Development of a plan and assistance in establishing treatment and social support services outside ACMHS
- Assistance in social and welfare issues impacting on Recovery including consumer advocacy
- Establishing engagement and empowering consumers to guide and participate in their own recovery
- One off assessment and diagnostic clarification
- Monitoring of recovery and providing 7-day follow-up post an inpatient admission.
- Managing relapse and crisis including intensive follow up with the aim of preventing hospitalisation
- Medication review
- Short term psychoeducation/Psychological therapy
- Support to carers and outside agencies involved in the consumer's care
- Discharge planning including liaison with primary health services and GP's
- Referral to outside agencies to continue care
- Referral to CTT within ACMHS if long term care coordination is required
- Ongoing monitoring and management of risk.

National Outcome and Case Mix Collection (NOCC)

Care Coordinators are required to collect measures for all active consumers on admission, clinical review and on discharge from ATT. Measures to be collected are:

- HoNoS (Health of the Nation Outcome Scales)
- LSP-16 (Life Skills Profile)
- Kessler-10 (K-10)

[NOCC Procedure](#)

Treatment Support Discharge Plans (TSDP)

A TSDP should be completed in collaboration with consumers and carers to reflect their recovery goals and how these will be achieved.

Each consumer will have a TSDP which will be guided by [AKG Treatment Support and Discharge Planning and Clinical Review of Care Procedure](#). Discharge planning will commence on admission. TSDPs will be completed within 14 days of ATT activation and developed in accordance with the following principles:

- Maximum consumer participation and, where possible, carer/family members – work in partnership to develop the TSDP
- Care planning to include the consumers perspective and recovery goals.
- Solution focused approach.
- Provides information on accessing tools including e-health information and supports to navigate the mental health system.

The TSDP should be signed by the consumer with a copy provided.

Physical Health Assessment

ATT is responsible for ensuring the consumer's physical health care needs are assessed on admission, and any physical health needs that are identified are addressed by the consumer's GP. This includes the management of chronic and preventative physical health care issues.

- Metabolic screening will occur at the initial assessment
- Consumers will be referred to their GP for a full physical health examination where indicated.
- Consumers with an identified GP will have a GP letter sent informing the GP of the ATT 10-week program.
- Consumers that do not have a GP will be supported in locating a GP within the ATT episode of care.

Physical health issues identified during admission to ATT must be managed in collaboration with the consumer/carer and their GP and follow up arranged with the GP. Physical health issues requiring immediate medical attention that cannot be managed in a timely manner by the consumer's GP should be escalated via local procedures to a conveniently located ED.

Physical health issues related to psychotropic medications prescribed during admission to ATT, or related to other mental health interventions or illness, may be managed by ATT clinicians, and must be communicated to the consumer's GP. ATT may also refer as appropriate to other services/programs and groups to support management of physical health with consent.

Guidance for ongoing monitoring and management must be provided in the consumer's discharge summary.

- [Metabolic Screening Policy](#)
- [Medication Profile](#)
- [Monitoring Neuroleptic Medication Side Effects](#)

Clinical Psychology

ATT Psychology provides diagnostic clarification and short-term psychological intervention with consumers. This can include psychoeducation regarding diagnosis, CBT/DBT based therapy and strategies to manage emotional dysregulation. This service also acts as an introduction to Clinical Psychology intervention and may aim at motivating the consumer to continue to seek psychological intervention in the community.

Alcohol and Other Drug Assessment

Co-occurring mental health and AoD needs are highly prevalent in the mental health consumer cohort. Consumers with co-occurring needs will be accepted into ATT provided their AoD use is not the primary reason for referral and will not interfere with the provision of clinical support to the point that ATT services will not be beneficial to the consumer.

AoD needs will be assessed during development of TSDP's.

ATT staff have the training and experience to provide interventions to address AoD needs however, where specialist skills are required, ATT staff will follow guidance from the [AKG Alcohol and Other Drugs – Patient Management Guideline](#) and seek advice and support from relevant Government (Next Step) and Non–Government Organisation (NGO) sector (e.g. Palmerston Association and the 12-Steps Program) for AoD services/assessment and/or intervention.

ACMHS encourages joint assessment and collaboration in the development of recovery plans with these services where possible.

Clinical Review

Consumers will have a clinical review of care after 5 weeks of activation with ATT. [AKG Treatment Support and Discharge Planning and Clinical Review of Care Procedure.](#)

Consumer non-engagement

Where a consumer did not attend (DNA) an appointment, the Care Coordinator and/or Doctor is responsible to follow-up to ensure safety of the consumer. A telephone call to the consumer and if necessary, NOK should be attempted on the day the consumer DNA's. Further follow-up including home visit may be necessary depending on the risk profile of the consumer.

[DNA procedure](#)

Consumers with multiple presentations to an Emergency Department (ED).

Active consumers of ATT who present to an ED on more than two occasions in any four-week period irrespective of the reason will have a review by their care coordinator and treating doctor.

Following presentation to AKG ED, the ED Liaison nurse will forward any documentation to ATT via ATTReferral.Armadale@health.wa.gov.au for the attention of Triage and CC noting the number of times that the consumer has presented in the last four weeks and requesting a review of the consumer and their TSPD.

The care coordinator or triage clinician will contact the consumer to review the reasons for presentation, ascertain current mental or physical state and risk, identifying any immediate intervention required. On review, the treating doctor and care coordinator will determine if a crisis management plan needs to be formulated in the TSDP. If an existing crisis management plan is in place this should be clearly reviewed and updated.

The care coordinator will meet with the consumer/carer/support(s) to discuss and agree on any crisis management plans.

In the event that an ED presentation is for physical health issues, the treating team will follow the above process to review if any psychiatric treatments may be relevant to the presentation.

If a consumer active with the AMHS presents to the AHS ED during office hours the following process will be followed: [Active Community ED business hours](#)

Discharge

Planning for discharge should commence at time of admission supporting contemporary best practice which is consumer-centred, flexible, strengths based, recovery focused and collaborative between all key stake holders.

Discharges from ATT can occur due to:

- Completion of treatment/intervention.
- Admission to a mental health inpatient facility.
- Disengagement by consumer.
- Consumer moving out of the catchment area.
- Treatment/intervention of more than 10 weeks is required.
- The consumer's whereabouts is unknown

Discharge from ATT can be to:

- CTT/EEP
- Primary care/alternative service provider/GP
- An ATT from another mental health service
- A mental health inpatient ward or CHRTT
- A correctional facility
- Discharge outright from Mental Health Services.

The following documentation is required for all discharges from ATT:

- Care Transfer Summary (SMHMR916) or letter to GP
- Mental Health Assessment (SMHMR902)
- Discharge RAMP
- Discharge NOCC measures.
- PSOLIS Referral outcome form
- Discharge TSDP
- Evidence of physical examination or request made to GP
- Care coordination Consumer Discharge Checklist

[Discharge and Transfer of Care](#)

Discharge to CTT

The ATT care coordinator will complete all required documentation for presentation at the Clinical intake Meeting.

Wherever possible transfer of care will include a face-to-face handover with the receiving CTT clinician using the ISoBAR handover standard.

If possible, a joint meeting with the consumer, carer, allocated ATT clinician and new CTT care coordinator may be arranged for the purpose of introduction and smooth transition of care.

A letter to the preferred GP of the consumer should be sent by the ATT treating doctor with information regarding current treatment regime and plans for transfer to a new team.

Clinical differences of opinion regarding transition pathways are to be escalated initially Consultant to Consultant, and if required through the Mental Health Medical Co-Director.

Operational barriers to transition should be escalated through the Team Leaders, and if required Program Manager - Community.

Discharge to Primary Care

ATT consumers may be referred back to their GP for ongoing management in keeping with recovery focused and least restrictive care. Before discharge the plan should be discussed in the Clinical intake meeting.

Clinical Handover:

A discharge letter including information regarding treatment and progress while within the service with recommendations regarding ongoing care should be completed by the treating doctor. Telephone contact with the GP regarding plans for discharge by the Care Coordinator and treating doctor is also encouraged.

Discharge to Other Community Mental Health Services

Consumers may be referred to another community mental health service if they move out of ACMHS catchment area while still requiring ATT or CTT services.

Clinical Handover:

The service who the consumer is to be transferred to, should be contacted by phone by the Care Coordinator to inform them of the transfer of care and provide information. A written referral should be forwarded to the accepting service via Email. It must include:

- Mental Health Assessment (SMHMR902)
- Care Transfer Summary (SMHMR916)
- Discharge RAMP
- Discharge TSDP

A letter to the consumer's preferred GP should be sent with information regarding current treatment regime and plans for discharge to a new team.

Admission to an Inpatient Mental Health Service

An admission to an inpatient ward may be required at any point during referral or treatment.

- All admissions will be in accordance with the processes/procedures contained within relevant policies and guidelines including the following:
[Admission of Community Mental Health Consumers to Inpatient Services Guideline](#)
- If admission is required for any consumer an ATT clinician will contact the Bed Flow Coordinator of Leschen Mental Health Unit AKG Hospital, stating the type of bed that is required and other relevant details to assist with bed allocation.
- An ATT clinician will provide a clinical handover to the receiving inpatient team including required handover documentation:
- The ATT clinician is also required to register the request for admission on the Enterprise Bed Management (EMB) system. This will enable the streamlining and standardisation of the bed allocation and management of patient transfers across the state (Inter Hospital Transfers).
- If the admission is to a secure unit under the Mental Health Act 2014 the care coordinator will be required to arrange for the transport of the consumer to the Mental Health Inpatient Unit. This may be via ED if a direct admission cannot be facilitated.
- The admitting ATT clinician will make all reasonable attempts to contact any family members, carers and/or other agencies identified and consented to by the consumer, to inform of the admission. Once the consumer is admitted they are discharged from ATT.
- When inpatient units experience high levels of acuity, referring community clinicians will explore alternative options and will treat patients more assertively in the community and accept early discharge from AKG MH inpatient services that can be managed assertively in the community. Refer to [EMHS BRAG Action Cards](#) and [EMHS Mental Health Patient Flow and Bed Capacity Framework](#) for roles and responsibilities.

Transporting a Consumer to Hospital Under the Mental Health Act

It is the responsibility of ATT to arrange and facilitate transport of a consumer under the Mental Health Act (MHA) to hospital when:

- The consumer has been assessed by ATT and placed under the MHA 2014.
- ATT have been handed over an outstanding transport to be completed after hours by other Clinical Teams of ACMHS.
- A GP has placed a consumer within AKG catchment under the MHA 2014 and has requested assistance to transport the consumer to a place of assessment.

The process is as follows:

- The Authorised Mental Practitioner or Doctor who completed the Form 1A must also complete a Form 4A.
- The Authorised Mental Practitioner or Doctor must complete the Mental Health Transport Risk Rating Form. [Patient Transport Risk Rating Form](#). This will inform whether Transport Officers and/or Police assistance is required. Refer to Metropolitan Mental Health Referral and Transport Order Process – Referrer Information Sheet [Flow Chart](#)
- The ATT clinician should give Police and/or Wilson Medic One the afterhours ATT mobile number for contact when they are able to complete the transport

- ATT clinician should attend with Police and/or Transport officer to inform and assist in the transport of the consumer.
- Forced entry can only be carried out by Police. If required police should be contacted.
- If the consumer is physically compromised, they should be transported by Wilson's Medic One or St. Johns ambulance.
- ATT clinicians are required to inform the accepting hospital of their arrival. This may be keeping in contact with the PLN of an ED department or the Bed Manager of an inpatient unit depending on the arrangements for admission.

It is expected that if the transportation is requested by another clinical team at AKG that they have completed the Form 4A and Mental Health Transport Risk Rating Form. They should also have liaised with the receiving hospital including providing the required documentation for admission and contacted Wilson's Medic One and/or Police before handing over to the ATT afterhours team.

After Hours Follow up Requests

If a consumer of AMHS requires follow-up afterhours their Care Coordinator, treating doctor or allocated clinician is required to provide formal handover both in writing and face to face to ATT afterhours clinicians.

They are required to:

- Provide a written ISOBAR handover in the form of completion of the ATT afterhours referral form [A/H referral form](#). Important information to include is:
 - Clear plan of requested intervention ensuring up to date demographic information.
 - Current medication regime and PRN options.
 - Alternative contact options with an identified escalation pathway if consumer cannot be contacted.
 - An alternative plan or escalation pathway should the first plan not be successful. For example, if a consumer doesn't answer the door, what should the team then do?
 - Risk assessment and Risk management Plan.
- Verbal handover to after-hours staff on duty after 4.00pm

Feedback to referrers will be provided by ATT via documentation in the DMR and PSOLIS, emailed & subsequently discussed at daily Stand-Up Meeting with care coordinator or allocated clinician.

Between the hours of 2200 - 0830 any mental health response will be coordinated through the Mental Health Emergency Response Line 1300 555 788

Escalation of Care

On recognition that a consumer's mental or physical health is deteriorating, it must be escalated to provide the most appropriate and timely intervention. Escalation should occur if the consumer:

- Shows deterioration in mental or physical health.
- Declines treatment and support
- Is at significant risk of injury to themselves or another person.
- Moves out of area/is not locatable.
- Requests an early review/carers concern.

The treating doctor, clinician/MDT will determine the planned intervention which may include but is not limited to:

- Increased contact/frequency of assessment
- Early Medical Review
- Mental Health Act Assessment
- Admission

Consumers who have No Fixed Abode (NFA)

There may be circumstances where a consumer engaged with ATT does not have a fixed address and is homeless within our catchment area. The consumer will not be disadvantaged if they become NFA status. In these circumstances the treating team will:

- Assist the consumer to attempt to access appropriate and suitable accommodation.
- Make efforts to maintain treatment if the environment is safe to do so.
- Make concerted efforts to maintain contact with the consumer to monitor and assess the consumers wellbeing.
- If a consumer becomes uncontactable the disengagement process will be followed.
- PSOLIS alerts will be utilised to communicate any delays in treatment or risk issues should the consumer present to another service.
- Each case will be discussed within the treating team so that all risk issues and last MSE can be considered to inform clinical decision making for consumers of concern.

Supervision and Line Management

ATT clinicians will be provided line management from a senior member of staff. Line supervision is to be provided monthly on operational matters including, but not limited to, performance, workload management and human resource related matters.

The following processes fall within the scope of line manager's responsibilities.

- Peak Performance Review; required to be undertaken annually in accordance with the [EMHS Peak Performance Policy](#)
- Performance Management; as outlined in the [WA Health managing Unsatisfactory and Substandard Performance Policy Framework](#),
- Fitness to work; as outline in the [EMHS Fitness for Work policy](#)

Inhouse training/education will reflect best practice that meets local demands, challenges, and needs, with consideration given to further improving skills and knowledge in teamwork, case formulation, communication, documentation, electronic record keeping and the model of service. ATT staff are required to attend mandatory skills assessment (MSA training day annually)

Clinical Supervision can be accessed to support clinicians in their professional environment with the goal of improving clinical practice and consumer outcomes.

Please refer to the following: [EMHS Clinical Supervision in MH Policy](#). There is a requirement for the staff member to complete a supervision contract agreement outlining the agreed goals, objectives, responsibilities, format, method, and model for clinical supervision.

Team Meetings

Daily Huddle

Occurs via MS Teams at 8.30 which includes ED, Consultation Liaison, Inpatient wards, CRHTT and all AHS community mental health teams.

The meeting includes patient flow between all areas of the mental health service and is attended by the rostered ATT triage clinician and a senior member of the team.

Stand Up Meeting

Occurs each weekday 8:45 and is the forum to discuss the day's business, risk, consumers of concern and staffing needs.

Intake and Allocation Meetings

Intake Meetings within Community Mental Health occur daily with members of the MDT present. The Team will review the referral information contacting the referrer if insufficient details are available.

The team will consider possible treatment requirements including assessment, medication, psychosocial needs, level of acuity and discipline specific requirements in identifying the most appropriate community program to meet the consumers current needs.

When allocating the consumer a care coordinator, clinician suitability including caseload acuity, skills and experience are considered to enable a best fit approach as per the [EMHS Care Coordination in MH Services Policy](#)

Business Meeting

Held monthly. This meeting is to discuss ATT operational matters, KPI's, policies, issues, staffing, policy/guidelines changes work and safety, facilities etc. Staff are invited to submit agenda items for discussion each month and are expected to attend.

Abbreviations and Acronyms

AHS	Armadale Health Service
AKG	Armadale Kalamunda Group
AMHS	Armadale Mental Health Service
ASSIST	Alcohol, Smoking, Substance Involvement Screening Tool
ATT	Assessment and Treatment Team
CTT	Clinical Treatment Team
CRHTT	Crisis Resolution Home Treatment Team
DNA	Did Not Attend
ED	Emergency Department
EMHS	East Metropolitan Health Service
FOC	First on Call
GP	General Practitioner
iSoBAR	Identify, situation, observations, background, agree plan, read back
LUNTERS	Liverpool University Neuroleptic Side Effect Rating Scale
MDT	Multi-Disciplinary Team
MHAF	Mental Health Assessment Form
MHLN	Mental Health Liaison Nurse
NOCC	National Outcomes and Case mix Collection
NUM	Nurse Unit Manager
PSOLIS	Psychiatric Services Online Information System
PSP	Personal Support Person
RAMP	Risk Assessment and Management Plan
TSDP	Treatment Support and Discharge Plan
TRS	Triage Rating Scale
WebPAS	Web based Patient Administration System

References/ Policy Context

Chief Psychiatrist of Western Australia

- [Chief Psychiatrist's Standards for Clinical Care](#)

HDWA

- [Enterprise Bed Management - Mental Health Business Rules](#)
- [WA Health MP 0095 Clinical Handover Policy](#)

- [Mental health Consumer Medication Information MP0069/17](#)
- [Triage to Discharge – Mental Health Framework for State-wide Standardised Clinical Documentation](#)

EMHS

- [Care Coordination in Mental Health](#)
- [EMHS Community/ home visiting Policy](#)
- [EMHS Patient Flow and Bed Capacity Framework](#)
- [EMHS Clinical Supervision in Mental Health Policy](#)

AKG

- [Admission of Patients from Community Mental Health Service to Armadale Mental Health Inpatient Units](#)
- [Assessment and Admission Documentation Requirements](#)
- [Assessment Treatment Team Process Manual](#)
- [Community Home Visiting Safety](#)
- [LUNSERS \(Liverpool University Neuroleptic Side Effect Rating Scale\) - using for monitoring Neuroleptic side](#)
- [Metabolic Screening](#)

Appendix A: Mental Health Triage Rating Scale – EMHS Community Services

Code/ description	Response type/ time to face-to- face contact	Typical presentations	MH Service action/ response	Additional actions to be considered
A Current actions endangering self or others	Emergency services response IMMEDIATE REFERRAL	- Overdose - Other medical emergency - Siege - Suicide attempt / serious self harm in progress - Violence / threats of violence and possession of weapon	Community clinician* notify ambulance, police and / or fire brigade. Contact fellow clinicians	Keeping caller on line until emergency services arrive Ascertain what assistance is required Notification of other relevant services (e.g. child protection)
B Very high risk of imminent harm to self or others	Very urgent MH response WITHIN 2 HOURS	- Acute suicidal ideation or risk of harm to others with a clear plan and means and / or history of self harm or aggression - Very high risk behaviour associated with perceptual / thought disturbance, delirium, dementia or impaired impulse control - Urgent assessment requested by Police	Face to face assessment required AND / OR Community clinician advice to attend a hospital emergency department when community clinician cannot attend in timeframe or ED treatment required	Providing or arranging support for consumer and / or carer while awaiting face-to-face MHS response (e.g. telephone support/therapy; alternative provider response). Telephone secondary consultation to other service provider while awaiting face-to-face MHS response. Advise caller to ring back if the situation changes. Arrange parental/carer supervision for a child/adolescent, where appropriate.
C High risk of harm to self or others and/or high distress, especially in absence of capable supports	Urgent MH response WITHIN 12 HOURS	- Suicidal ideation with no plan and/or history of suicidal ideation - Rapidly increasing symptoms of psychosis and/or severe mood disorder - High risk behaviour associated with perceptual/thought disturbance, delirium, dementia, or impaired impulse control - Unable to care for self or dependants or perform activities of daily living - Known consumer requiring urgent intervention to prevent or contain relapse	Community clinician face-to-face assessment within 8 hours AND Community clinician telephone follow-up within ONE HOUR of triage contact.	As above Obtaining corroborating/additional information from relevant others.
D Moderate risk of harm and/or significant distress	Semi-urgent MH response WITHIN 48 HOURS	- Significant client/carer distress associated with serious mental health illness (including mood/anxiety disorder) but not suicidal - Early symptoms of psychosis - Requires priority face-to-face assessment in order to clarify diagnostic status - Known consumer requiring priority treatment or review.	Community clinician face-to-face assessment	As above
E Low risk of harm in short term or moderate risk with high support/stabilising factors.	Non-urgent MH response Within 2 weeks	- Requires specialist mental health assessment but is stable and at low risk of harm in waiting period - Other service providers able to manage the person until MHS appointment (with or without MHS phone support) - Known consumer requiring non-urgent review, treatment or follow-up	Continuing care or equivalent. Face-to-face assessment	As above
F Referral: not requiring face-to- face response from MHS in this instance	Requires further triage contact/ follow up	- Other service (e.g. GPs, private mental health practitioners) more appropriate to person's current needs - Symptoms of mild to moderate depressive, anxiety,	Community clinician to provide formal or informal referral to an alternative service provider or advice to attend a particular	Facilitating appointment with alternative provider (subject to consent/privacy requirements), especially if alternative intervention is time critical



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